

Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°135/MVE-BDBV/26/09/2026

Report date: 26 September 2026

Publication date: September 27, 2026



HIGHLIGHTS (last 24 hours)

A total of 78 new confirmed cases of Ebola Virus Disease (EVD) were reported, including 34 community deaths. The confirmed cases were distributed across the provinces of Ituri (57 cases), North Kivu (17 cases), Haut-Uélé (3 cases), and Bas-Uélé (1 case). In addition, 15 deaths were recorded in Ebola Treatment Centers (ETCs).

A total of 422 new samples were analyzed, 78 of these samples were found to be positive for the Bundibugyo virus, representing 18.5% positivity.

✓ No new health zones have been affected in the last 24 hours.

ACCUMULATIVE CAS 8 067	ACCUMULATIVE DEATH CONFIRMED 3 901	LETHALITY 48,4 %	PATIENTS IN ISOLATION/CTE 773	ACCUMULATIVE HEALED 2 070	TRACKING RATES CONTACTS (Of the Day) 74,7 %
---	---	-----------------------------------	--	--	--



Affected provinces and affected health zones



7 Provinces affected

Uele, Ituri, North Kivu,
South Kivu, Tshopo, Lower Uele and
Sud Ubangi



63 out of 167
health zones affected
(37,7%)

Summary of key points from the last 24 hours

For the day of September 26, seventy-eight (78) new confirmed cases

of Ebola Virus Disease (EVD), of which **34 community deaths** have been recorded. The confirmed cases came from the provinces of Ituri (57 cases), North Kivu (17 cases), Haut-Uélé (3 cases) and Bas-Uélé (1 case).

Over the past 24 hours, a significant increase in reported confirmed cases has been observed in Ituri province, particularly in the **Mandima health zone** (9 cases vs. 25 cases: **177.8%**). This zone accounts for **43.9%** of new confirmed cases and **74.1%** of community deaths recorded in the province.

In addition, **15 deaths were recorded in the CTEs** (13 in Ituri, 1 in North Kivu and 1 in Haut-Uélé), bringing the total number of confirmed deaths to 49 at the close of the day.

Thirty-seven patients (17 in North Kivu, 14 in Ituri, 5 in Haut-Uélé and 1 in Bas-Uélé) were declared cured after obtaining two negative results for Ebola virus disease.

As of September 26, 2026, the DRC had recorded **8,067 confirmed cases, including 3,901 deaths**, representing a case fatality rate of **48.4 %**. Ituri province remained the epicenter, concentrating **76.3%** of reported confirmed cases and **73.1%** of new confirmed cases.

No new health zones have been affected in the last 24 hours. Thus, **63 out of 167 health zones (37.7%)** have been affected in **7 provinces** since the start of the epidemic. Ituri has 28 out of 36 affected health zones, followed by North Kivu (16/34), Haut-Uélé (7/13), Tshopo (7/23), Bas-Uélé (3/11), South Kivu (1/34) and South Ubangi (1/16).

Table 1. Distribution of confirmed cases and deaths by affected province, as of September 26, 2026.

Province	New confirmed cases (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	57	6155	2 834	46,0%	28/36 (77,8 %)
North Kivu	17	1523	906	59,5%	16/34 (47,1 %)
Haut-Uélé	3	331	139	42,0%	7/13 (53,8 %)
Boss	0	43	16	37,2%	7/23 (30,4 %)
South Kivu	0	3	1	33,3%	1/34 (2,9 %)
Bas-Uélé	1	10	4	40,0%	3/11 (27,3%)
Sud Ubangi	0	2	1	50,0%	1/16 (6,3%)
Total	78	8 067	3 901	48,4%	63/167 (37,7 %)

Table 2. Distribution of confirmed cases and deaths by province and zone as of September 26, 2026

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Cas (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	6155	2834	46,0%	57	27	13	40
Give	11	1	9,1%				0
Undersized	8	6	75,0%				0
Ariwara	8	2	25,0%				0
Aungba	12	5	41,7%				0
Bamboo	201	46	22,9%				0
have	2	2	100,0%				0
Bunia	1697	543	32,0%	5	0	5	5
Ladies	40	9	22,5%				0
Drodre	15	7	46,7%				0
Fireworks	82	31	37,8%				0
Gethy	7	3	42,9%				0
Flounder	2	0	0,0%				0
Kilo	35	12	34,3%	1	0	0	0
Team	166	100	60,2%	4	2	2	4
Liter	283	140	49,5%	1	0	0	0
Logo	13	5	38,5%				0
Fight	33	8	24,2%				0
distributed	4	1	25,0%				0
Washing	27	9	33,3%				0
Darkness	118	65	55,1%	25	20	0	20
Mangala	346	153	44,2%				0
Mongbwalu	697	328	47,1%	0	0	2	2
Nanny	284	159	56,0%	7	5	2	7
Nizi	771	277	35,9%	6	0	1	1
Legumes	127	37	29,1%				0
Jungle	12	4	33,3%				0

Rwampara	1078	363	33,7%	8	0	1	1
Tchomia	76	25	32,9%				0
A valves	THAT	493	THAT				0
North Kivu	1523	906	59,5%	17	4	1	5
Me	329	220	66,9%	6	4	0	4
Farm	18	7	38,9%				0
Butembo	256	165	64,5%				0
Ten	1	0	0,0%				0
Kalunguta	51	16	31,4%	1	0	0	0
Cut	555	353	63,6%	2	0	1	1
Source	1	0	0,0%				0
Kyondo	33	20	60,6%				0
Lubero	21	11	52,4%				0
Be careful.	31	11	35,5%	6	0	0	0
Manguredjipa	13	5	38,5%				0
Masereka	28	9	32,1%				0
The museums	121	65	53,7%				0
Mutwanga	12	2	16,7%	1	0	0	0
Oicha	24	14	58,3%	1	0	0	0
Vuh	29	8	27,6%				0
Haut-Uélé	331	139	42,0%	3	3	1	4
Kill Mangbetu	39	17	43,6%	0	0	1	1
Dumb	1	1	100,0%				0
Gombari	3	1	33,3%				0
Mathematics	100	43	43,0%				0
Lining	77	36	46,8%	1	1	0	1
Beans	2	1	50,0%				0
Ordinary	109	40	36,7%	2	2	0	2
Boss	43	16	37,2%	0	0	0	0
Bafwasende	7	1	14,3%				0
Kabondo	5	1	20,0%				0
Lubunga	1	0	0,0%				0
Maquis-							
Kisangani	23	10	43,5%				0
Mangobo	5	3	60,0%				0
Boss	1	0	0,0%				0
Wanie-Rukula	1	1	100,0%				0
South Kivu	3	1	33,3%	0	0	0	0
Miti-Murhesa	3	1	33,3%				0
Bas Uélé	10	4	40,0%	1	0	0	0
Blind	1	1	100,0%				0
Vladana	6	1	16,7%	1	0	0	0
Walk	3	2	66,7%				0
Sud Ubangi	2	1	50,0%	0	0	0	0
Hair	2	1	50,0%				0
Total	8067	3901	48,4%	78	34	15	49

*These are the confirmed deaths that occurred in the various CTEs in the province which are being broken down to be classified by health zone.

NA : Not Applicable.

Table 3. Status of alerts notified by province for September 26, 2026

DPS	Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CTE/CT (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	48	8	56	9	3	36	0	12	9
Bas-Uélé	79	5	84	8	5	71	0	13	0
Ituri	1 168	78	1246	119	77	822	0	179	99
North Kivu	800	45	845	84	43	704	2	127	62
Sud Ubangi	9	0	9	1	0	0	0	1	0
Boss	99	1	100	6	0	94	0	6	2
Total General	2 203	137	2 340	227	128	1 727	2	338	172

Figure 1. Evolution of cases by date of symptom onset over the last 21 days

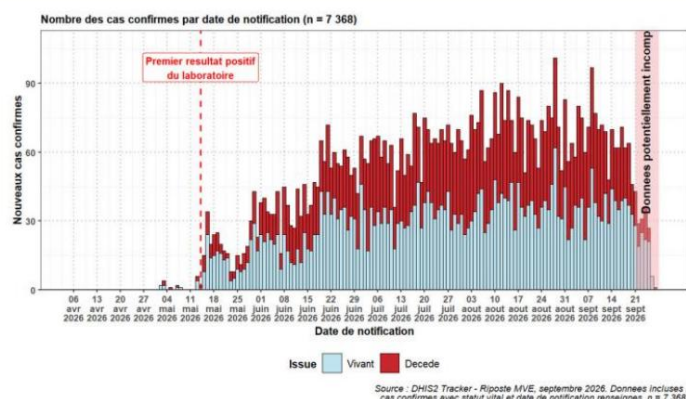
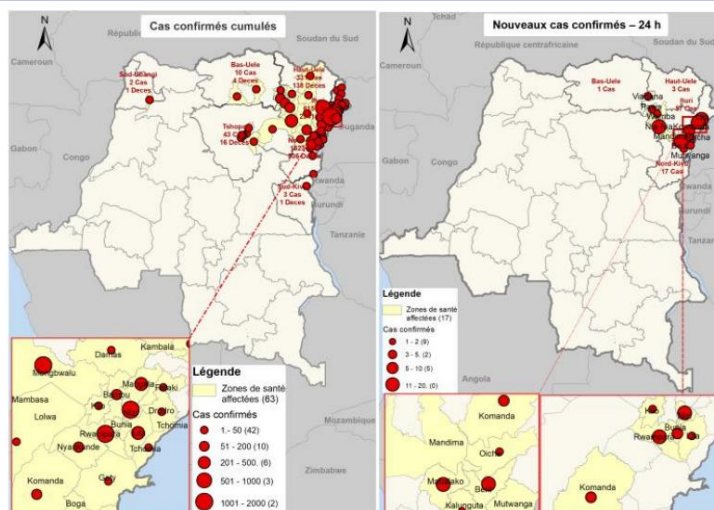


Figure 2. Spatial distribution of confirmed cases (24 hours and cumulative) by health zone.



1. COORDINATION

1.1. Main Actions

- Support for response activities in the affected provinces through the technical pillars.

2. RESPONSE ACTIONS BY PILLAR

2.1. Epidemiological surveillance

2.1.1. Main Actions

In total, **2,340 alerts** were reported during the day of September 26, 2026.

2,084 (89.1%) were verified. Of these, **355 alerts were validated as suspected cases** and **338 (95.2%)** were investigated, of which **172** were transferred to CT/CTE (Table 3).

The follow-up proportion as of September 26, 2026 was **74.7% (23,187/31,034)**, lower than that of the previous day (77.8%). This proportion was 88.8% (87/98) in South Ubangi, 86.6% (123/142) in Bas-Uélé, 79.1% (778/984) in Haut Uélé, 75.7% (11,819/15,611) in North Kivu, 73.2% (9,829/13,424) in Ituri and 71.1% (551/775) in Tshopo.

2.2. Update on checkpoint and point of entry (PoE/PoC) activities

2.2.1. Main actions

For September 26, 2026, four (4) alerts were notified to the various PoE/PoCs, including three (3) in Ituri and one (1) in North Kivu. In Ituri, an alert intercepted at the Mudzi Bala PoC (Bunia Health Zone) was validated and transferred to the Bunia CTE; another

A live person intercepted at the Mabakese PoC (Mambasa Health Zone) was identified, but the suspect refused transfer to the CTE. A body arriving from Uganda, intercepted at the Dele PoC (Rwampara Health Zone), could not be swabbed due to a lack of cooperation from the security team. In North Kivu, a traveler intercepted at the Maboya PoC (Kalunguta Health Zone) was identified, investigated, and referred to the Tinge Hospital.

Table 4. Status of key PoE/PoC indicators by province, as of September 26, 2026

Indicators	Ituri	North Kivu	Boss	Global
Report completeness	98,3% (59/60)	95,2% (20/21)	100% (18/18)	98,0% (97/99)
% of functional strategic PoE/PoC	100%	100%	100%	100%
Number of people who have passed PoE/PoC	144 996	79 509	20 878	245 383
% of travelers screened	97,0%	97,7%	98,8%	97,4%
% of travelers who washed their hands	94,9%	97,7%	99,0%	96,1%
% of travelers made aware	99,5%	97,7%	100,0%	99,0%
Number of alerts notified	3	1	0	4
Number of validated live alerts (suspected cases)	2	1	0	3
Number of bodies intercepted today % of	1	0	0	1
suspected cases transferred to CTE/CT	50,0%	100,0%	THAT	66,7%
% of lifeless bodies swabbed/secured	0,0%	THAT	THAT	0,0%
% of suspected cases investigated within 4 h	100,0%	100,0%	THAT	100,0%

2.2.2. Challenges

In Ituri, low transfer of validated suspects (50.0%; 1/2) following a refusal, unswabbed dead body at the Dele PoC (0/1), proportion of hand washing below target (94.9%), Avakubi PoC non-functional due to insecurity, low encoding of alerts in DHIS2 Tracker and insufficient mobility in the Rwampara and Bunia health zones; in North Kivu, armed attack at the Vulemba/Butuhe PoC (Kalunguta health zone) which cost the life of a service provider, declining completeness (95.2%; 20/21) and late handling of alerts by alert cells.

2.3. Laboratory

2.3.1. Main actions

- **Ituri: 57 new positive results** (30 living and 27 deaths) out of 212 new samples received and analyzed (**positivity: 26.9%**).
- **North Kivu: 17 new positive results** (13 living and 4 dead) out of 179 samples received and analyzed (**positivity: 9.5%**).
- **Haut-Uélé: 3 new positive results** (3 deaths) out of 9 samples received and analyzed (**positivity: 33.3%**).
- **Bas-Uélé: 1 new positive result (1 live)** out of 15 samples received and analyzed (**positivity: 6.7%**).
- **Tshopo: 7 samples received and analyzed** (7 live) all tested negative.

2.4. Infection Prevention and Control (IPC/EDS)

2.4.1. Main actions

- In North Kivu, 21 out of 25 rings were opened (84.0%), 7 out of 22 households were decontaminated (31.8%), and 17 out of 18 health facilities (94.4%); 1 health facility received infection prevention and control (IPC) kits, 9 were assessed, 69 were monitored, and 121 providers were briefed, with 116 handwashing stations monitored and 759 [unclear - possibly "

liters of water were provided in public places, with a promptness and completeness rate of 37.5% (6/16 health zones). Regarding blood donations, 40 alerts were received, 39 bodies were swabbed, and 35 blood donations were carried out (87.5%); one failed swab was recorded in Katwa due to strong community resistance.

- **In Tshopo**, 17 health facilities were monitored in 6 health zones, and 198 healthcare providers were briefed on standard precautions (hand hygiene, injection safety, in-hospital monitoring, and preparation of chlorine solutions); 10 health facilities were equipped with infection prevention and control (IPC) kits, 21 handwashing stations were monitored, and 13 triage units were installed. Cumulatively, 89.3% of households that hosted a confirmed case (25/28) and all of the affected health facilities (33/33) were decontaminated within 48 hours.
- **In Bas-Uélé**, 2 rings were opened and the 2 households around the confirmed cases in Viadana were decontaminated (100%); 11 ESSs were monitored in Ganga (5/5), Viadana (3/3) and Buta (3/3), the 3 triage units in Buta were inspected and 32 service providers were briefed, including 8 out of 12 (66.7%) in Buta on temperature checks using a contactless thermometer; 25 members of the management teams from 5 health zones received integrated training with the support of FHI 360. As for EDS, 6 bodies were swabbed without EDS being carried out; training of service providers on the generalities of Ebola Virus Disease as well as on standard precautions, including 5 MCZs, 5 MDHs, 5 SEHAs, 5 ACs and 5 laboratory technicians from 5 health zones (Bili, Monga, Likati, Aketi and Bondo) with the support of FHI360; monitoring and support of healthcare providers in 11 social and solidarity economy (SSE) facilities across 3 health zones.

2.4.2. Challenges

In North Kivu : low household decontamination (31.8%; 7/22), no IPC kits provided to households of cases (0/7), low promptness and completeness of reports (37.5%; 6/16), average IPC score of 30.4% in the assessed ESSs of Musienene and Mabalako, community resistance leading to an EDS failure in Katwa, non-alignment of 86.6% of pillar members on the motivation list and need to activate decontamination teams in Biena, Masereka, Kyondo, Mabalako and Vuhovi. **In Tshopo** : insufficient community IPC inputs (ICP/WASH kits delivered to 40.0% of households in cases; 20/50), only 2 out of 99 ESS (2.0%) with an IPC score above 80%, insufficient trained PPL and incinerator at Makiso-Kisangani HGR out of order.

2.5. Holistic care

2.5.1. Main actions

- **In Ituri**, 73 new admissions were recorded during the day, compared to 77 discharges, including 14 recoveries. Thus, 380 patients are hospitalized out of 1015 beds (occupancy rate of 37.4%); 360 out of 375 patients (96%) received hot meals, and children under LNPE/SLM were fully covered at the CTE (9/9), at the nursery (9/9), and in the community (33/33); 39 patients are in critical condition.
- **In North Kivu**, 54 new admissions were recorded, compared to 76 discharges, including 17 recoveries; 328 patients (97 confirmed and 231 suspected) are hospitalized, including 223 in Ebola Treatment Centers (ETCs) out of 354 beds, representing an occupancy rate of 63.0%, with the remaining 105 (32.0%) being treated in Integrated Health and Social Services (ESS); 16 patients are in critical condition at the Beni ETC. A total of 142 patients, 116 caregivers, and 336 frontline healthcare providers received hot meals.
- **In Haut-Uélé**, 11 new admissions were recorded against 18 discharges (5 recovered); 60 patients are in isolation out of 138 beds (occupancy rate of 43.5%).
- **In Bas-Uélé**, 1 new admission (1 confirmed at the CT of Limba/Viadana) was recorded against 3 discharges (1 recovered in Ganga and 2 non-cases in Buta); 5 patients (1 confirmed and 4 suspected) remain in isolation in the non-standard CTs of Limba, Poko and Buta.

2.5.2. Challenges

Care still partially substandard and limited capacity: In Ituri, the Ebola Treatment Centers (ETCs) in Komanda, Kilo, Mandima, and Lolwa have limited capacity; 18 patients were without food in Lolwa (7), Mambasa (6), and Mandima (5), and ambulance movements were poorly reported. In North Kivu, 32.0% of hospitalized patients (105/328) were treated outside of ETCs, with high in-hospital mortality (39%) and missing data for Mabalako and Mutwanga. In Bas-Uélé, there is no standardized ETC and no ambulance for patient transfers.

2.6. Vaccination

2.6.1. Main actions

- **In Tshopo**, the ERVEBO vaccine shortage persists (0 frozen doses at the hub depot) despite strong demand from the population and local populations since the death of a doctor on September 24; only 47 thawed doses remain available in the health zones, with a complete shortage in Kabondo, Tshopo, and Bafwasende. The provincial total has reached 4,953 local populations and vulnerable populations vaccinated, representing 42.3% of the target (11,703), including 2,446 in Makiso-Kisangani, 643 in Kabondo, 550 in Tshopo, 540 in Mangobo, 500 in Bafwasende, and 274 in Lubunga.
- **In Ituri**, Ebola vaccination continued in the Bunia health zone with MSF support: 58 people were vaccinated in the last 24 hours, representing a daily coverage of 19.3% (58/300). These vaccinated individuals included 20 women (34.5%) and 38 men (65.5%), including 1 pregnant woman and 4 breastfeeding women; no adverse events following immunization (AEFIs), severe or non-severe, were reported. The cumulative total has reached 436 people vaccinated (183 women and 253 men).

2.6.2. Challenges

Total stockout of ERVEBO vaccine in Tshopo (0 frozen doses; 47 thawed doses remaining) in the face of high demand from the population and PPL, with a clear stockout in 3 ZS (Kabondo, Tshopo and Bafwasende); coverage still partial of the provincial target (42.3%), low in Lubunga (16.6%; 274/1,646) and in Mangobo (23.4%; 540/2,305); in Ituri, low daily coverage (19.3%); vaccination of PPL not yet started in North Kivu; no data from Bas-Uélé and Haut-Uélé.

2.7. Continuity of Care

2.7.1. Main actions • In Tshopo,

evaluation of the ECC/Makiso Medical Centre, development of the recovery plan for the SIMAMA CRP and the VITA NOVA CM, evaluation of the PCSSE work plan (3rd week of September) and monitoring of PPLs at the CTM and the CTE; the 20 agents of the visited ESS are vaccinated (100%), including 13 PPLs.

- **In Bas-Uélé**, Evaluation of the ESS of Rubi Lisanga (ZS of Buta) on routine vaccination and specific programs (HIV, tuberculosis and leprosy), revealing a drop in ANC1 coverage from 100% in July to 58% in August 2026.

2.7.2. Challenges

Inadequate triage and isolation areas and reduced use of services, particularly maternity, in the ESS visited in Tshopo; in Bas-Uélé, low attendance at ANC (58% in August) and loss to follow-up of routine vaccination in Rubi Lisanga.

2.8. Risk Communication and Community Engagement (RCCE)

2.8.1. Main actions

- **In Ituri**, 9,937 home visits reached 35,228 people, 1,552 educational talks were held (18,377 people), 36 community debates (1,084 people) and 161 mass awareness campaigns (23,232 people), 4 advocacy campaigns were conducted and 23 spots were broadcast on 36 radio stations; 280 community alerts (28 deaths) were notified and 293 feedbacks out of 320 (91.6%) were resolved within 48 hours, as well as 11 incidents out of 13 (84.6%); 10 community relays and 40 traditional healers conducted a guided tour of the Rwampara CTE.
- **In North Kivu**, CREC teams carried out 8,914 home visits (25,662 people), organized 156 educational talks (3,643 people) and 38 mass communication sessions (5,012 people) as well as 8 briefings (199 people); the teams also Notified 134 out of 650 expected alerts (20.6%), resolved 9 out of 11 feedback points (81.8%), and 2 incidents

out of 5 (40.0%), and engaged 1,153 people out of 4,750 (24.3%); media coverage of testimonials from 14 recovered patients discharged from CTE/CT in Katwa and Biena.

- **In Tshopo**, CREC teams visited approximately 234 households, held 4 community dialogues (Bafwasende, Wanie-Rukula and Kabondo) and hosted 4 interactive broadcasts, reaching 2,211 people (978 men and 1,233 women); they notified and dealt with 24 community alerts, resolved 2 feedbacks (100.0%) and lifted 3 resistances (100.0%).
- **In Bas-Uélé**, 15 home visits (95 people), 1 educational talk (27 people) and 1 mass awareness campaign (28 taxi drivers from the ATADI association) were carried out in Ganga, and an awareness campaign was conducted in the household of the confirmed case of Limba to overcome resistance to contact tracing.

2.9. Mental Health and Psychosocial Support (MHPSS)

2.9.1. Main actions

- **In North Kivu**, 16 confirmed cases (100%), 234 confirmed cases under follow-up, 234 confirmed cases under follow-up, 169 new suspected cases, and 346 suspected cases under follow-up have benefited from interventions. SMSPS; 66 households and 74 PPLs were supported, 8 recovered out of 16 followed up in the community (50.0%), 103 results announced including 17 positive results, and 424 people (147 men, 208 women, and 69 children) benefited from psychoeducational sessions
- **In Ituri**, 14 of 31 confirmed cases (45.2%), 96 confirmed cases under follow-up out of 138 (69.6%), 31 new suspected cases out of 65 (47.7%) and 93 suspected cases under follow-up out of 146 (63.7%) were taken care of; 387 accompanying persons and 43 PPLs were supported, 14 recovered out of 21 followed in the community (66.7%), 58 results announced including 14 positive cases and 16 children followed in daycare (100%).
- **In Haut-Uélé**, 7 newly registered confirmed cases, 27 confirmed cases under follow-up and 15 suspected cases benefited from SMSPS interventions, as well as 62 accompanying persons, 40 PPLs and 33 household members; 18 results announced, including 9 positive ones.

2.9.2. Challenges

- **In North Kivu**, armed men raided the PoC of Vulemba (AS of Butuhe, ZS of Kalunguta) and opened fire on a contractor assigned to screening, who died on the spot.

2.10. Logistics

2.10.1. Main actions

- **In North Kivu**, construction work on the huts at the Mususa PoC (Musienene Health Zone) was completed, 1,000 liters of water were supplied to the Kyaghala PoC, and a vehicle and 200 liters of fuel were provided to the Laboratory pillar by FHI 360; work on the Musienene CTE continues.
- Support for the mobility of response teams; in **Tshopo**, monitoring of the construction of the CTE in Bafwasende, provision of a survival kit and a hygiene kit to the family of a recovered patient discharged in Bafwandambo and support for the mobility of the pillars; **In Bas-Uélé**, continuation of the construction of the transit center of the HGR in Buta and rehabilitation of the DPS meeting room by the WHO, with a limited stock of chlorine (25 kg) and fuel (80 litres).

2.11. Security

2.11.1. Main actions

- **North Kivu**, around 12:25, armed men raided the PoC of Vulemba (AS of Butuhe, ZS of Kalunguta) and killed a screening contractor; the Risk Mitigation and Security pillar provided escort for EDS teams in Katwa, Butembo and Beni (100%) and updated the mapping of recommended accommodation sites for stakeholders in Beni and Butembo.

2.12. PSEA (Prevention of Sexual Exploitation and Abuse)

2.12.1. Main actions

In Ituri, 38 participants (10 women and 28 men), including WHO drivers, CDC and AFNET, as well as IPC hygienists, were briefed on PSEA in Rwampara and signed the code of conduct, and 393 people (48 women, 58 men, 161 girls and 126 boys) were sensitized in Bunia, Rwampara and Aru; in Tshopo, 13 enumerators were briefed in Bafwasende for EAS data collection and 64 community members (including 16 women and 32 young people) were consulted in Lindi and Kenge; in Haut-Uélé, 19 people were sensitized in the Tely and Nsele health districts in Isiro.

2.12.2. Challenges

- Still insufficient coverage of PSEA activities, with only 41 of the 119 health zones monitored (34.5%) aware of the reporting mechanism; not all areas affected by focal points are covered and there is insufficient mass awareness materials and code of good conduct.

MESSAGES CLES

- Active and persistent community transmission is observed in the provinces of Ituri, North Kivu, Haut-Uélé, Tshopo and Bas-Uélé. Over the past 24 hours, a significant increase (**83.9%**) in the notification of confirmed cases was observed in the province of Ituri, which is particularly marked in **the Mandima health zone** (9 cases vs 25 cases: **177.8%**) and accounts for **43.9%** of the new confirmed cases as well as **74.1%** of community deaths recorded in this province.
- However, progress is still needed in the area of Epidemiological investigations: **95.2%** of suspects were investigated within 24 hours of notification. However, the contact tracing rate remains fluctuating with a slight decrease compared to the previous day (**74.7% vs 77.8 %**).
- Strengthening priority interventions (surveillance, care, IPC, logistics and community engagement) remains essential to rapidly interrupt transmission chains: Rapidly increase reception capacities in ETC/CT in North Kivu and strengthen cross-border surveillance with South Sudan.
- Strengthen preparedness and response to Ebola Virus Disease-B in the provinces of the River Corridor and surrounding areas.

TIMELINE OF KEY EVENTS

- From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.
- Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of funds.



TASK FORCE PRESIDENTIAL EBOLA 17
Ministry of Public Health, Hygiene and Social Welfare
National Institute of Public Health
Public Health Emergency Operations Center
MVE17 Incident Management System

Director General of INSP

Dr. MWAMBA KAZADI Dieudonné

Email: dieudonne.mwamba@insp.cd

Incident Manager

Professor Dr. AHUKA MUNDEKE Steve

Email: steve.ahuka@unikin.ac.cd

Operations Manager

Dr. KOBBE NGWAMA John

Editorial Team

Professor Dr. MULANGU KATULUMBA Félix-Médard

Dr. KATUSI BISELENGE Jules

Mrs. WISE DIE Marie

Dr. DIUR MANANASHI Floryan

Mr. Kamuina Kebela John

Contact :

www.insp.cd

contact@sgiebola17.org

Freephone number: 151. www.sante.gouv.cd